

NOSE SYMPTOMS

Give urgent attention to the patient with nose symptoms and:

- Head injury with clear watery discharge from nose → 18.

Refer urgently.

Approach to the patient with nose symptoms not needing urgent attention

Manage according to nose symptom/s:

Blocked/runny nose
Ask about duration and associated symptoms:

Sore throat or fever

Acute viral infection likely

(like common cold, influenza or COVID-19)

- If temperature $\geq 38^{\circ}\text{C}$, chills or body pain, **influenza** or **COVID-19** more likely.
 - Assess for COVID-19 → 40.
- Advise to wear mask indoors and social distance. While unwell, avoid contact with elderly/those with chronic diseases/groups, wash hands.
- Give **paracetamol** 1g 4-6 hourly (up to 4g in 24 hours) as needed for up to 5 days.
- Explain antibiotics are not needed. Advise to rest and maintain hydration.
- Advise to return if worsening symptoms: if cough/difficulty breathing → 38, if face pain → 32, if ear pain → 33.

Pain when pushing on forehead/cheeks, headache worse on bending forward, recent common cold

Sinusitis likely

- Give **paracetamol** 1g 4-6 hourly (up to 4g in 24 hours) as needed for up to 5 days.
- Give **sodium chloride 0.9%** nose drops as needed.
- Give **oxymetazoline 0.05%** 2 drops in each nostril 8 hourly as needed for a maximum of 5 days. Advise against overuse which may worsen blocked nose.
- If symptoms ≥ 10 days, fever $\geq 38^{\circ}\text{C}$, purulent discharge, face pain ≥ 3 days, or symptoms worsen after initial improvement, give **amoxicillin** 500mg 8 hourly for 5 days. If severe penicillin allergy¹, give instead **azithromycin** 500mg daily for 3 days.
- If recurrent, test for HIV → 110.
- If poor response to antibiotic, refer.
- Refer same day if:
 - Tooth infection
 - Swelling over sinus or around eye
 - Neck stiffness

Recurrent sneezing or itchy/runny/blocked nose most days for > 4 weeks. May have itchy eyes, ears or throat.

Allergic rhinitis likely

- Help to identify and advise to avoid triggers².
- Give **fluticasone**³ nasal spray 100mcg (1 spray) in each nostril twice a day. Advise patient to aim nozzle outwards and upwards and avoid sniffing vigorously.
- Give **chlorphenamine** 4mg 6-8 hourly as needed for up to 5 days only when symptoms worsen (side effect is sedation).
- If nose very blocked at night, give **oxymetazoline 0.05%** 2 drops in each nostril at night for a maximum of 5 days. Advise against overuse which may worsen blocked nose.
- If recurrent eye problem, exclude allergic conjunctivitis → 31.
- If recurrent skin problem, exclude urticaria and eczema → 67.
- If recurrent cough or wheeze, exclude asthma → 123.
- Review after 3 months: if symptoms still not controlled despite good adherence to nasal spray, add **cetirizine** 10mg at night.
- If symptoms severe and persist despite treatment, refer.

Runny nose with persistent cough, and/or frequent throat clearing

Upper airway cough syndrome (postnasal drip) likely

- Check throat: secretions or cobblestone appearance may be seen at back of throat.
- Treat as for **allergic rhinitis** (see adjacent column).
- If no improvement after 2 weeks, refer/discuss.

Bleeding nose

- Firmly pinch nostrils together for 10 minutes with patient sitting and leaning forward.
- Check BP:
 - If $< 90/60$, give **sodium chloride 0.9%** 1L IV rapidly, repeat until systolic BP > 90 . Continue 1L 6 hourly. Stop if breathing worsens.
 - If $\geq 140/90$ → 132.
- If still bleeding, insert **bismuth iodoform paraffin paste (BIPP)** soaked ribbon gauze into nostril/s:
 - If bleeding stops, advise to return next day to remove BIPP gauze.
 - If bleeding persists, refer urgently.
- If patient on aspirin or warfarin, doctor to review medication and if on warfarin, check INR.
- Advise to avoid nose-picking and contact sport if recurrent bleeds.
- If continually rubbing or itchy nose, consider allergic rhinitis (see adjacent).
- If recurrent bleeds and no improvement with above management, refer.

¹History of anaphylaxis, urticaria or angioedema. ²Common triggers include pollens, household pets, house dust mite, cockroaches and moulds. ³If on lopinavir/ritonavir or atazanavir/ritonavir, avoid fluticasone, discuss/refer instead.